



You may already know that your baby will arrive **via cesarean section** (C-section), or you may be wondering what would happen if you were told during labor that you could need surgery. Knowing what C-sections involve will **help you prepare mentally** and practically for **what lies ahead** for both you and your baby.

# Cesarean section

## The decision to operate

Around a third of all babies in the US and a quarter of all babies in the UK are born surgically. The vast majority of C-sections are done for medical reasons, of which many are known in advance, such as placenta previa, a multiple birth, or a breech position. In these cases, you will have an “elective,” or planned, C-section.

Roughly one in eight births in the UK is an “emergency,” or unplanned, C-section. Some vaginal births can develop complications that mean this is the safest option for you and your baby. However, not all situations are life threatening: a long, slow labor is the most common reason given for unplanned surgical delivery. Your medical team will always talk through the reasons why a C-section is being recommended. These discussions are open to your partner, too, but it is for you to decide and consent to. For some women, a C-section may come as a relief, whereas others may feel a sense of disappointment or failure at not having a vaginal birth. These are normal responses; asking questions about the risks and benefits will help you to make a decision you are happy with.

You can request a C-section without a medical reason if you want, and this should be set out in your birth plan (see pp.88-9), but it is important to be fully aware of the risks and recovery implications, which are greater on

balance than those for a vaginal birth. Your doctor will explain the pros and cons so that you can make a fully informed decision.

## What to expect from a cesarean section

Although having a C-section is major surgery, it can be a life-saving one. Like any surgery, it can carry risks. For the mother this may mean increased blood loss (both during and after surgery), cardiac arrest, infection of the uterus or uterine lining, bladder damage, complications for future pregnancies, greater risk of future hysterectomy, and a longer hospital stay.

Your baby may experience more stress, because you won't have the benefit of the labor hormones that calm the baby. She may be born with residual fluid in the lungs and nasal passages that make independent breathing more difficult. There is a small risk (about 2 percent) of your baby receiving a nick or cut, but this will heal within a few days. There is a possibility that your baby could be admitted to the neonatal intensive care unit (see p.310).

Before you are discharged, you'll be able to discuss the surgery and ask any further questions about aftercare or possible complications. Your recovery will take about six weeks so it may be a good idea to get help taking care of your baby at first if you can.